TEST TEST
MRN : **A1351656026**
Birthday : **2011-11-24**
Phone : **5555555555**
Visited on: 2024 Nov 15 13:05 (Age at visit: 13 years)
Electronically signed by: Teresa Asam-Jensen, MD on 2024-11-19 11:16 AM

CC

Est care: evaluate mid-cyle pain

HPI

DL is a pleasant 37 yo G6P6 woman seen in IL via telemedicine with synchronous video for mid-cylce pain. Reports regular cycles, 25-26 days long, tracking for about 12 years. Has noted pain in the left or right lower pelvis around CD 13-14 with bloating for the last 18 months, starting with return of cycles after her 6th child. Pain described as sharp, worse with activity and high impact. Occurs every cycle, like clockwork for the last 18 months roughly. Describes persistent, all-day deep pain that is responsive to deep pressure -could worsen or become somewhat better No other GI sx: no n/v/d/flatus or belching. Stable intensity and length (2-3 days). Not worsening. No evaluation to date. May improve with heat, message, occasional acetaminophen 600mg once daily if needed.
Last delivery: NSVD (long labor, 40 hrs). Healthy pregnancy but severe nausea.
typical cycle: +BB 2d after; no midcycle bleeding. Occasionally very extensive menstrual bleeding. No intensive menstrual pain- worse midcycle. No menstrual GI sx. Occasionally very short cycles.
No medications then or now.
PMH no other dx
PSH: no pelvic surgeries or other surgeries

PMHx

no dx pmh

PSHx

Wisdom teeth removed
no

FHx

mother - alive, has Gestational DM

Soc Hx

married

Ob Preg Hx

Pregnancy History - G6, T0, P0, A0, L6

ROS

12 pt ROS performed including Gen, HEENT, Resp, CV, GI, Skin - nothing additional to HPI

Medications

No known medications

Allergies

No known allergies

Mental/Functional

Normal mood; no c/o significant PMS mood shifts

Vitals

Height/Length	Weight	BMI	Blood Pressure	Temperature	Pulse	RR	SpO2
5' 5"	**135 lbs 0 oz**	**22.46**				**16**	

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Height/Length	Weight	BMI	Blood Pressure	Temperature	Pulse	RR	SpO2

Recorded: 2024 Nov 15 13:35 by: Teresa Asam-Jensen

EXAM

GEN: NAD, sitting comfortably facing screen, engaged easily in conversation. Appears well.
HEENT: N/C. AT. MMM to observation. Symmetric facies without edema.
NECK: supple, NT, FROM
RESP: nonlabored breathing, no use of accessory of muscles of respiration
DERM: skin warm and dry; no rashes or lesions to observation
EXT: symmetric movement bilat/UE; no cyanosis/clubbing/edema
NEURO: AO x 3, grossly normal coordination and tone
PSYCH: judgment/insight intact, euthymic mood/ congruent affect; good eye contact

Chart Review:
11/11 - current
4d menses, 1d BB

10/10/15 - 11/10
3d menses + 2d TEBB
6d mucus cycle (MCS 17.2)
+pain CD 13-14 - no midcycle bleeding
PPP 11 days, no PM BB or spotting

No labs or imaging to review.

Assessment

Abnormal menstrual cycle (N92.6) Irregular menstruation, unspecified modified 15 Nov, 2024

Lower abdominal pain (R10.30) Lower abdominal pain, unspecified modified 15 Nov, 2024

(Problem reconciliation performed by Teresa Asam-Jensen 01:40 PM 15 Nov 2024 Last updated)

Plan

Abnormal menstrual cycle

Clinical history of new onset midcycle pelvic pain without bleeding in context of menstrul bleeding irregularities (tail-end brown bleeding, occasionally prolonged bleeding): differential includes recurrent ovarian cysts, ovarian mass, PCOS and endometriosis; much less likely scar tissue from prior (undx) pelvic infx.

Irregularities in cycle length suggest contribution of hormonal instability from either PCOS or chronic inflammation: differential includes abnormal follicular phase estradiol levels or luteal phase defects (low progesterone OR abnormal progesterone:estradiol relationship)

Recommendations:

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- start with routine follow up appointment for pelvic/pap with ob-gyne and pelvic ultrasound. This can be done through ob-gyne or an independent imaging - target midcycle days around mucus peak day to attempt to capture abnormalities contributing to pain
US PELVIS transabd/TV sent.
- evaluate estradiol levels in follicular phase leading up to and including midcycle: QOD from (mucus) peak day -4 through pk+2: labs sent to QUEST
- evaluate post-peak (luteal phase) progesterone/estradiol curve: pk+3 through 11 inclusive
LABS/STUDIES:
Standing order: x5
(Ordered by Teresa Asam-Jensen on 15 Nov, 2024 at 12:42 PM)
Estradiol (Ordered by Teresa Asam-Jensen on 15 Nov, 2024 at 12:42 PM)

LABS/STUDIES:
(Ordered by Teresa Asam-Jensen on 19 Nov, 2024 at 10:09 AM)
PROGESTERONE (Ordered by Teresa Asam-Jensen on 19 Nov, 2024 at 10:09 AM)
ESTRADIOL (Ordered by Teresa Asam-Jensen on 19 Nov, 2024 at 10:09 AM)

Lower abdominal pain
Differential and work up as above. Limitation of pain to 1-2 days of midcycle - occurring on either side of the abdomen but not simultaneously - with lack of GI sx strongly supports a gynecologic cause of pain. Nonetheless, if all imaging and labs are normal, follow up CT or MRI of abd/pelvis would be appropriate to assess for masses from any site, endometriosis or other abnormalities.
Discussed that if the evaluation shows no clear abnormalities OR any imaging findings are concerning for masses, scarring, large cysts or endometriosis, consultation with NaPro surgeon would be advised.
Pt v/u and appreciation - all questions answered.

45 min visit
Follow up: beginning of cycle AFTER lab results are complete

Goals
Dx and treat midcycle pain

Health Concerns
None